Cash for the Prevention of Acute Malnutrition (C4PX): Setting the Transfer Value for a Nutritional Top Up in Myanmar

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Results and overview

Table 1 presents a range of potential starting point values to assist in deciding transfer amounts for a nutritional top up to household cash assistance for prevention of malnutrition under consideration in Rakhine and Sagaing states of Myanmar.

Table 1. Starting point values for estimating a nutritional top up value for prevention of malnutrition (all values in MMK)

Region	Assumed household assistance transfer values	Estimated starting point transfer values per day for prevention		
		Child 6-23 months	Child 6-59 months	PBWG
Rakhine	1606 daily (or 50000 monthly per person)	374	1099	2433
Rakhine	1830 daily (or 57000 monthly per person)	317	1024	2263
Rakhine	2216 daily (or 69000 monthly per person)	217	896	1971

Values are provided under three scenarios of potential household assistance values to be provided, in each region.

The calculation involved four steps which seek to answer the following questions:
1) What is the cost of meeting the nutrient needs of targeted beneficiaries using locally available and commonly consumed foods?
2) How much money do households spend on food on average?
3) What is the gap between diet costs and what households can meet themselves?
4) What proportion of the remaining diet cost is met by existing household cash assistance?

The method seeks to ensure that if households own economic capacity or the amount provided through existing household cash assistance are low relative to the cost of meeting nutrient needs, the resulting top up value will be relatively larger.

The value produced using the method should broadly reflect the cost meeting the nutrient needs of beneficiaries while accounting for households own economic capacity and household cash assistance.

When a single beneficiary per household is targeted, it is recommended to use the value which reflects the cost of meeting the nutrient needs of a PBWG, as we would expect breastfeeding to continue after 6 months, which has additional dietary costs for the mother. If there are additional transfers amounts provided for additional children in the household, then it is proposed the cost of meeting the nutrient needs of a child can be used.

While the approach can provide a starting point value for deciding transfer values, the final values received by beneficiaries will heavily depend on considerations which are secondary to the value calculated, such as the number of beneficiaries per households, the provision of a flat rate or differing values for women or

The method seeks to ensure that if households own economic capacity or the amount provided through existing household cash assistance are low relative to the cost of meeting nutrient needs, the resulting top up value will be relatively larger.

Data for the calculation came from the following sources outlined in table 2.

Table 2. Data used in the calculation and sources.

Data	Value & Sources
Food price data	Full details on foods included in the analysis, and the sources are included in table 3. Two sources: 1) WFP food prices
Household assistance transfer per person/ day 3 values modelled: 50,000; 57,000; 69,000 MMK	MEB_Technical_Note_CWG_May2024_Final
Percentage of transfer spent on food 65%	MEB_Technical_Note_CWG_May2024_Final
Value of the food portion of the MEB 2334 MMK	MEB_Technical_Note_CWG_May2024_Final
Percentage of MEB value households can meet themselves 30%	MEB_Technical_Note_CWG_May2024_Final
Coverage of supplementation & breastfeeding rates	Global Nutrition report: breastfeeding up until 2 years: >65% Women with anc iron supplementation: >55 % Ministry of Health and Sports. 2017. Public health statistics report (2014-2016). Myanmar. https://mohs.gov.mm/Main/content/publication/public-health-statistics-report-2014-2016

Detailed methodology:

1) What is the cost of meeting the nutrient needs of targeted beneficiaries using locally available and commonly consumed foods?

For this step ENHANCE software was used to calculate the lowest cost combination of foods which could meet the nutrient needs of beneficiaries. Food prices were available for 43 foods. Of these 25 commonly consumed, nutritious foods were chosen in line with WFP guidance on calculating a transfer value for prevention of acute malnutrition, which recommends 25 foods with a minimum from relevant foods groups (see table 3).

Foods included were as follows:

Rice; Potato; Noodle; rice; Noodle, wheat; Cowpea; Peanut; Roselle ; Green leaf (Dark tha lon ywet);

; Green leaf (Su poke ywet); Okra; gourd leaf; Pumpkin; Long bean (pe taung shay); Papaya; Banana

; Coconut; Carrot, red; Fish paste; Fish, small, fry; Fish dried; Beef; Chicken; Egg; Milk, cow.

It is strongly recommended that food prices continue to be monitored and the number of markets with monitoring be expanded considerably in areas where cash is used a modality for prevention of malnutrition.

1) How much money do households spend on food on average?

The Economic Capacity to Meet Essential Needs (ECMEN) analysis assesses the economic capacity of households to meet basic needs was used as an estimate of households own economic resources.

The proportion of expenditure on food was presumed to be 65%. This was presumed based upon the food MEB as a proportion of the total MEB, which reflects food expenditure as a proportion of overall expenditure in the population.

2) What is the gap between diet costs and what households can meet themselves?

Households own estimated food expenditure was then subtracted from diet costs calculated int step 1.

4) What proportion of the remaining diet cost is met by existing household cash assistance?

Three different potential household transfer values were used to estimate this see table 1, to reflect potential values which may be used for household transfer values.

Further considerations in deciding final values.

The approach outlined above provides a starting point for deciding a transfer value when using an individual nutritional cash top up to support prevention of acute malnutrition. However, the final values received by beneficiaries will heavily depend on considerations which are secondary to the value calculated. Outlined below are some other key factors to consider.

- *How many individuals should receive a transfer per household?*

In some cases, it may only be feasible to provide a transfer value for one individual per household, i.e., a "flat rate" or a transfer may be provided for each additional eligible individual in the household. Where the target group is children aged 6-59 months, it is very likely that there may be more than one eligible child per household. If a flat rate is provided, the money available of the programme will be diminished for larger households.

- *Should a flat rate be provided for all individuals or different values for PBWG or children?*

While the cost of meeting the nutrient needs of children is lower than PBWGs, we should be mindful that breastfeeding should continue beyond 6 months, and has associated elevated dietary costs for women too. If two separate values are provided, one for a child and one for a PBWG, this would require changing, and likely reducing, the value when the child turns six months, depending on beneficiary eligibility criteria. This would have adverse effects on the mother and child and is not generally advised. However, if the separate transfers and values are provided for more than one child per household, the transfer value for the child may be applicable.

- *When and how often should the value be updated (key in high inflation contexts)?*

Monitoring the cost of meeting nutrient needs and adjusting the transfer value accordingly is essential. Including all, or some key indicator, foods included in the calculation of the transfer value in existing price monitoring is recommended. Values can be reviewed to assess whether values should be adjusted on a periodic basis, in line with the MEB/ household assistance values, or separately, or sporadically in response to food price inflation.

- How do to account for intra-household sharing of the transfer?

Some level of sharing of the transfer is likely. In in-kind prevention programmes, we often in food insecure contexts presume some level of intra-household sharing of SNF. The above approach for transfer value calculation does not take potential household sharing into account.

It is likely that there will be some level of undernutrition among households which WFP targets for prevention activities (e.g., micronutrient deficiencies). This can increase nutrient needs and associated costs. Where deemed necessary and feasible, values can be adjusted upwards to reflect these issues, where resources allow.

- *How to handle when transfer values are set by other actors or systems or are limited by resources and differ from the values derived from the nutritional top up calculation?*

If the values set by other actors, such as through government national social protection systems, or low due to resource constraints, linkages to nutrition specific programming such as micronutrient supplementation, and specialised nutritious foods should be strongly considered, particularly where transfer values are considerably below those estimated through the above approach.

If the values provided are so as to be unlikely to enable necessary improvements in the dietary intake of beneficiaries, the programme objective may also need to be reconsidered. For instance, the programme.

- *How to manage programme costs associated with transfer values, and relatedly, the breadth versus depth of assistance provided?*

In any programme, a key consideration is cost, and relatedly, the breadth versus depth of assistance provided i.e., targeting the most vulnerable people with more, or a greater number of vulnerable people with less. Ultimately, the values provided should reflect an amount which is feasible to give in light of the COs targeting strategy, while still reflecting the cost of meeting nutrient needs of beneficiaries, and programme objectives.

The size of the transfer relative to the household assistance may also be an important consideration, as if the value is far larger than the household assistance, this could create tensions in the community, and potentially a perverse incentive for beneficiaries. Noting that WFP household assistance provided should be sufficient to meet most or all of the identified needs of beneficiary communities which they cannot meet themselves or through other forms of assistance. If the level of assistance is very low compared to diet costs, this may be a cause for concern and should be investigated.

Annex 1: Potential number and types of food for expanded food price monitoring.

Table 3. Number and types of foods to consider for inclusion in expanded food price monitoring.

Food Group (suggested number of items to include)	**Specific foods items to consider**
Starchy Staples (4)	Staples chosen will be context dependent, but may include maize grains or meal, wheat flour, rice, cassava/gari, sorghum or millet etc. It is vital the list includes the most commonly consumed, cheapest variety staples.
Pulses, nuts, seeds (2)	Examples of some commonly consumed legumes are white, red and black beans, pigeon, split and chickpeas, lentils, soybean, bambara groundnuts. Nuts/seeds: Ground nuts, Sesame seeds
Vegetables & Fruits (10)	Of particular interest, where widely consumed are: Dark green leafy vegetables Other cruciferous vegetables Other widely consumed, low-cost fruits (e.g., banana) Orange fleshed vegetables and fruits. *Onion, tomato, and sweet corn, while often widely consumed, are of less interest from a nutrition standpoint.* Dried leaves such as baobab, or moringa/drumstick leaf, should only be included where they are commonly consumed in reasonable quantity on a regular basis.
Animal source foods (7)	Of particular interest, where widely consumed are eggs, dairy, dried fish, if not widely consumed can be substituted by other foods from the animal source food group.
Meats (2)	Of particular interest, where widely consumed are red meats such as goat, beef, mutton, or pork or camel. Poultry meats, while less nutrient dense, may be included where more widely consumed. Offal meats should only be included where they are commonly consumed in reasonable quantity on a regular basis.
Fish (2)	Including preserved fish where widely consumed, such as dried, salted or smoked fish. Small fish can also often be an important food source for poorer households.
Eggs (1)	Eggs, where commonly consumed, they should be considered for inclusion.
Dairy (2)	This would likely include fresh, fermented, evaporated and/or powdered milk products.